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Passport No :

LABORATORY TEST REPORT

Patient Information	Sample Information	Client/Location Information
Name : **Lyubochka Svetka**	**Lab Id : 02232160XXXX**	Client Name : Sterling Accuris Buddy
Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:09 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : EDTA Blood	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Complete Blood Count

Test		Result	Unit	Biological Ref. Interval
Hemoglobin	Colorimetric	14.5	g/dL	13.0 - 16.5
RBC Count	Electrical impedance	4.79	million/cmm	4.5 - 5.5
Hematocrit	Calculated	43.3	%	40 - 49
MCV	Derived	90.3	fL	83 - 101
MCH	Calculated	30.2	pg	27.1 - 32.5
MCHC	Calculated	33.4	g/dL	32.5 - 36.7
RDW CV	Calculated	13.60	%	11.6 - 14

Total WBC and Differential Count

WBC Count	SF Cube cell analysis	H **10570**	**/cmm**	4000 - 10000

Differential Count					Absolute Count		
Neutrophils	Microscopic	73	%	40 - 80	**7716**	**/cmm**	2000 - 6700
Lymphocytes	Microscopic	**19**	**%**	20 - 40	2008	/cmm	1100 - 3300
Eosinophils	Microscopic	02	%	1 - 6	211	/cmm	00 - 400
Monocytes	Microscopic	06	%	2 - 10	634	/cmm	200 - 700
Basophils	Microscopic	00	%	0 - 2	0	/cmm	0 - 100

Platelet Count	Electrical impedance	150000	/cmm	150000 - 410000
MPV	Calculated	H **14.00**	**fL**	7.5 - 10.3

Peripheral Smear Examination

RBC Morphology	Normochromic Normocytic
WBC Morphology	WBCs Series Shows Normal Morphology
Platelets Morphology	Platelets are adequate with normal morphology.
Parasites	Malarial parasite is not detected.

Erythrocyte Sedimentation Rate

ESR	Capillary photometry	7	mm/1hr	0 - 14

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Sex/Age	: **Male / 41 Y** 01-Feb-1982	Registration on	: 20-Feb-2023 09:10	Location	:
Ref. Id	:	Collected at	: non SAWPL	Approved on	: 20-Feb-2023 13:33 Status : Final
Ref. By	:	Collected on	: 20-Feb-2023 08:53	Printed On	: 28-Feb-2023 10:26
		Sample Type	: EDTA Blood, Serum	Process At	: 1. NRL SAWPL Gujarat Ahmedabad Paldi

Blood Group

Test	Result	Unit	Biological Ref. Interval
ABO Type	"A"		
Rh (D) Type	Positive		

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Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:29 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Lipid Profile

Test	Result	Unit	Biological Ref. Interval
Cholesterol *Cholesterol oxidase – Peroxidase method*	189.0	mg/dL	Desirable : <200 Borderline High : 200-239 High : >240
Triglyceride *Ezymatic (Lipase/GK/GPO/POD)*	H **168.0**	**mg/dL**	**Normal : <150** **Borderline : 150-199** **High : 200-499** **Very High : >500**
HDL Cholesterol *PTA/MgCl2*	60.0	mg/dL	Low: <40.0 High: >60.0
Direct LDL *Direct measured*	H **100.39**	**mg/dL**	**Optimal: <100** **Near to above Optimal: 100–129** **Borderline High: 130-159** **High: 160–189** **Very High: =190**
VLDL *Calculated*	33.60	mg/dL	15 - 35
CHOL/HDL Ratio *Calculated*	3.1		Up to 5.0
LDL/HDL Ratio *Calculated*	1.7		Up to 3.5

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Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:45 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Fluoride plasma	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Biochemistry

Test	Result	Unit	Biological Ref. Interval
Fasting Blood Sugar *GOD-POD*	H **141.0**	**mg/dL**	**74 - 106**

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Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : EDTA Blood	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

HbA1c (Glycosylated Hemoglobin)

Test	Result	Unit	Biological Ref. Interval
HbA1c *High Performance Liquid Chromatography*	H **7.10**	**%**	**For Screening:** **Diabetes: >6.5%** **Pre-Diabetes: 5.7% - 6.4%** **Non-Diabetes: < 5.7%** **For Diabetic Patient:** **Poor Control : > 7.0 %** **Good Control : 6.0-7.0 %**
Mean Blood Glucose *Calculated*	157.07	mg/dL	

Explanation:-

- Total haemoglobin A1 c is continuously synthesized in the red blood cell throught its 120 days life span. The concentration of HBA1c in the cell reflects the average blood glucose concentration it encounters.
- The level of HBA1c increases proportionately in patients with uncontrolled diabetes. It reflects the average blood glucose concentration over an extended time period and remains unaffected by short-term fluctuations in blood glucose levels.
- The measurement of HbA1c can serve as a convenient test for evaluating the adequacy of diabetic control and in preventing various diabetic complications. Because the average half life of a red blood cell is sixty days, HbA1c has been accepted as a measurement which reflects the mean daily blood glucose concentration, better than fasting blood glucose determination, and the degree of carbohydrate imbalance over the preceding two months.
- It may also provide a better index of control of the diabetic patient without resorting to glucose loading procedures.

HbA1c assay Interferences:
Erroneous values might be obtained from samples with abnormally elevated quantities of other Haemoglobins as a result of either their simultaneous elution with HbA1c (HbF) or differences in their glycation from that of HbA (HbS).

Reference: ADA Guideline 2023

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Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:38 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Thyroid Function Test

Test	Result	Unit	Biological Ref. Interval
T3 - Triiodothyronine *Chemiluminescence*	1.01	ng/mL	0.58 - 1.59
T4 - Thyroxine	7.84	mg/mL	4.87 - 11.72
TSH - Thyroid Stimulating Hormone *Chemiluminescence*	0.8199	microIU/mL	0.35 - 4.94

TSH	T3/FT3	T4/FT4	Suggested Interpretation for the Thyroid Function Tests Pattern
Within Range	Decreased	Within Range	- Isolated Low T3-often seen in elderly & associated Non-Thyroidal illness. In elderly the drop in T3 level can be upto 25%
Raised	Within Range	Within Range	- Isolated High TSH especially in the range of 4.7 to 15 mIU/ml is commonly associated with physiological & Biological TSH Variability. - Subclinical Autoimmune Hypothyroidism - Intermitted T4 therapy for hypothyroidism - Recovery phase after Non-Thyroidal illness
Raised	Decreased	Decreased	- Chronic autoimmune Thyroiditis - Post thyroidectomy, Post radioiodine - Hypothyroid phase of transient thyroiditis
Raised or Within Range	Raised	Raised or Within range	- Interfering antibodies to thyroid hormones (anti-TPO antibodies) - Intermittent T4 therapy or T4 overdose - Drug interference-Amiodarone, Heparin, Beta blockers, steroids. anti-epileptics
Decreased	Raised or within Range	Raised or within Range	- Isolated Low TSH - especially in the range of 0.1 to 0.4 often seen in elderly & associated with Non-Thyroidal illness - Subclinical Hyperthyroidism - Thyroxine ingestion
Decreased	Decreased	Decreased	- Central Hypothyroidism - Non-Thyroidal illness - Recent treatment for Hyperthyroidism (TSH remains suppressed)
Decreased	Raised	Raised	- Primary Hyperthyroidism (Graves disease), Multinodular goitre Toxic nodule - Transient thyroiditis:Postpartum, Silent (lymphocytic), Postviral (granulomatous, subacute, DeQuervain'a) Gestational thyrotoxicosis with hyperemesis gravidarum
Decreased or within range	Raised	Within Range	- T3 toxicosis - Non-Thyroidal illness

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Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 12:40 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Urine	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Biochemistry

Test	Result	Unit	Biological Ref. Interval
Microalbumin (per urine volume) *Immunoturbidimetric*	10.50	mg/L	< 16.7

In random urine specimens, normal urinary albumin excretion is below 17 mg/g creatinine for males and below 25 mg/g creatinine for females.(3)
Microalbuminuria is defined as an albumin:creatinine ratio of 17 to 299 for males and 25 to 299 for females.
A ratio of albumin:creatinine of 300 or higher is indicative of overt proteinuria.
Due to biologic variability, positive results should be confirmed by a second, first-morning random or 24-hour timed urine specimen. If there is discrepancy, a third specimen is recommended. When 2 out of 3 results are in the microalbuminuria range, this is evidence for incipient nephropathy and warrants increased efforts at glucose control, blood pressure control, and institution of therapy with an angiotensin-converting-enzyme (ACE) inhibitor (if the patient can tolerate it).

Reference :

1. Bennett PH, Haffner S, Kasiske BL, et al: Screening and management of microalbuminuria in patients with diabetes mellitus: recommendations to the Scientific Advisory Board of the National Kidney Foundation from an ad hoc committee of the Council on Diabetes Mellitus of the National Kidney Foundation. Am J Kidney Dis 1995;25:107-112
2. Krolewski AS, Laffel LM, Krolewski M, et al: Glycosylated hemoglobin and the risk of microalbuminuria in patients with insulin-dependent diabetes mellitus. N Engl J Med 1995;332:1251-1255
3. Zelmanovitz T, Gross JL, Oliveira JR, et al: The receiver operating characteristics curve in the evaluation of a random urine specimen as a screening test for diabetic nephropathy. Diabetes Care 1997;20:516-519

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Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:41 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Protein

Test	Result	Unit	Biological Ref. Interval
Total Protein *Copper tartrate to colour complex*	7.00	g/dL	6.3 - 8.2
Albumin *Bromocresol Green Method*	4.20	g/dL	3.5 - 5.0
Globulin *Calculated*	2.80	g/dL	2.3 - 3.5
A/G Ratio *Calculated*	1.50		1.3 - 1.7

Bilirubin

Total Bilirubin *Azobilirubin chromophores*	0.70	mg/dL	0.2 - 1.3
Conjugated Bilirubin *Cationic Mordant Binding*	0.30	mg/dL	0.0 - 0.3
Unconjugated Bilirubin *Cationic Mordant Binding*	0.20	mg/dL	0.0 - 1.1
Delta Bilirubin *Calculated*	0.20	mg/dL	0.0 - 0.2

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Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:29 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Iron Studies

Test	Result	Unit	Biological Ref. Interval
Iron *Pyridyl azo Dye*	103.00	micro g/dL	49 - 181
Total Iron Binding Capacity (TIBC)	352.00		261 - 462
Transferrin Saturation *Calculated*	29.26	%	Children : >16 Adult : 20 - 50

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Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test	Result	Unit	Biological Ref. Interval
Homocysteine, Serum *Chemiluminescence*	H **23.86**	**micromol/L**	**6.0 - 14.8**

Summary and Uses:

- Total Hcy is a thiol-containing amino acid, produced by the intracellular demethylation of methionine to cysteine.
- Elevated levels of t Hcy may be used to exclude or confirm deficiencies of vitamin B12 or folate.
- It is recommended to test in patients using medications that interfere with folate status (methotrexate, antiepileptics), vegetarians without B12 supplementations, unexplained anemia, peripheral neuropathy or myleopathy, recurrent spontaneous abortions or infertility.
- Testing also recommended for patients 40 years of age with coronary artery disease to exclude homocystinuria.
- Elevations in tHcy levels have also been used as an independent risk factor of coronary or cerebral vascular disease. Treatment of moderate hyperhomocystinemia with folic acid supplementation for primary and secondary cardiovascular protection has met with inconsistent results and at present cannot be routinely recommended.

Limitations:

- The plasma must be seprated immediately on collection to avoid continuous synthesis of Hcy by red cells.
- Samples must be immediately stored on ice and serum centrifuged immediately before a complete clot is formed.
- Certain drugs, such as anticonvulsants, methotrexate, or nitrous oxide, may interfere with the assay.
- Cigarette smoking and coffee consumption increase tHcy levels.
- Intraindividual variability is approximately 8%; it can be as much as 25% in patients with hyperhomocystinemia.
- Generally, a single measurement of tHcy is considered adequate.

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Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Biochemistry

Test		Result	Unit	Biological Ref. Interval
Creatinine, Serum *Creatinine Amidohydrolase*		0.83	mg/dL	0.66 - 1.25
Urea *Urease, Colorimetric*	L	**18.0**	**mg/dL**	**19.3 - 43.0**
Blood Urea Nitrogen *Calculated*	L	**8.41**	**mg/dL**	**9.0 - 20.0**
Uric Acid *Uricase*		4.90	mg/dL	3.5 - 8.5
Calcium *Arsenazo III*		9.10	mg/dL	8.4 - 10.2
SGPT *UV with P5P, IFCC*		48.0	U/L	0 - 50
SGOT *UV with P5P*		27.0	U/L	17 - 59

Electrolytes

Sodium (Na+) *Direct- ISE*		143.00	mmol/L	136 - 145
Potassium (K+) *Direct- ISE*		4.90	mmol/L	3.5 - 5.1
Chloride (Cl-) *Direct- ISE*		105.0	mmol/L	98 - 107

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Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test	Result	Unit	Biological Ref. Interval
25(OH) Vitamin D *CLIA*	**8.98**	**ng/mL**	**Deficiency : <10** **Insufficiency : 10 - 30** **Sufficiency : 30 - 100** **Toxicity : >100**

Vitamin D is a fat soluble vitamin and exists in two main forms as cholecalciferol(vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight exposure & Ergocalciferol(vitamin D2) present mainly in dietary sources.Both cholecalciferol & Ergocalciferol are converted to 25(OH)vitamin D in liver.

Interpretation:

Increased In

- Vitamin D intoxication
- Excessive exposure to sunlight

Decreased In

- Malabsorption
- Steatorrhea
- Dietary osteomalacia, anticonvulsant osteomalacia
- Biliary and portal cirrhosis
- Thyrotoxicosis
- Pancreatic insufficiency
- Celiac disease
- Rickets
- Alzheimer disease

Limitations:

More recently, it has become clear that receptors for vitamin D are present in a wide variety of cells and that this hormone has biologic effects extending beyond the control of mineral metabolism. Vitamin D deficiency is not clear. Levels needed to prevent rickets and osteomalacia (15 ng/mL) are lower than those that dramatically suppress parathyroid hormone levels (20–30 ng/mL). In turn, those levels are lower than levels needed to optimize intestinal calcium absorption (34 ng/mL). Neuromuscular peak performance is associated with levels approximately 38 ng/mL. A recent study states that increasing mean baseline levels from 29 to 38 ng/mL was associated with a 50% lower risk for colon cancer and levels of 52 ng/mL with a 50% reduction in the incidence of breast cancer. It is recommended to have clinical correlation with serum 25(OH)vitamin D, serum calcium, serum PTH & serum alkaline phosphatase.

Dhote

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National Reference Laboratory: 101 to 109, First Floor, Sankalp Square II, Nr. Old Sharda Mandir Cross Road, B/s. Jalaram Mandir Railway Crossing, Ellisbridge, Ahmedabad, Gujarat- 380006, **Ph.: 812 813 0000**
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MC-2202

sterling ACCURIS Pathology lab that cares

Passport No :

LABORATORY TEST REPORT

Patient Information	Sample Information	Client/Location Information
Name : **Lyubochka Svetka**	**Lab Id : 02232160XXXX**	Client Name : Sterling Accuris Buddy
Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 12:04 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test	Result	Unit	Biological Ref. Interval
Vitamin B12 *CLIA*	L **< 148**	**pg/mL**	**187 - 833**

Vitamin B12 is essential in DNA synthesis, hematopoiesis, and CNS integrity.

Interpretation:

- **Increased In** : Chronic granulocytic leukemia , COPD and Chronic renal failure , Leukocytosis , Liver cell damage (hepatitis, cirrhosis) , Obesity and Severe CHF , Polycythemia vera , Protein malnutrition.
- **Decreased In** : Abnormalities of cobalamin transport or metabolism , Bacterial overgrowth , Crohn disease , Dietary deficiency (e.g. in vegetarians) , Diphyllobothrium (fish tapeworm) infestation , Gastric or small intestine surgery , Hypochlorhydria , Inflammatory bowel diseas , Intestinal malabsorption and Intrinsic factor deficiency

Limitations:

- Drugs such as chloral hydrate increase vitamin B12 levels. On the other hand , alcohol, aminosalicylic acid, anticonvulsants, ascorbic acid, cholestyramine, cimetidine, colchicines, metformin, neomycin, oral contraceptives, ranitidine, and triamterene decrease vitamin B12 levels.
- The evaluation of macrocytic anemia requires measurements of both vitamin B12 and folate levels; ideally they should be measured simultaneously.
- Specimen collection soon after blood transfusion can falsely increase vitamin B12 levels.
- Patients taking vitamin B12 supplementation may have misleading results.
- A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12. The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

Dhote

DR.TEJASWINI DHOTE
M.D. Pathology

Dr. Sanjeev Shah
MD Path

Dr.Yash Shah
MD Path

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Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:38 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test	Result	Unit	Biological Ref. Interval
PSA-Prostate Specific Antigen, Total	0.573	ng/mL	0 - 4

PSA is a glycoprotein that is expresses by both normal and neoplastic prostate tissue and is prostate tissue specific and not prostate cancer specific. PSA is constantly expressed in nearly all prastate cancers, although its level of expression on a percell basis is lower than in normal prostate epithelium. The absolute value of serum PSA is useful for determining the extent of prostate cancer and assessing the response to prostate cancer treatment; its use as a screening method to detect prostate cancer is also common.

Interpretation

Increased in

- Prostate disease (Cancer, Prostatitis, Benign prostatic hyperplasia, Acute urinary retention)
- Manipulations (Cystoscopy, Needle biopsy, Radiation therapy, Indwelling catheter, Prostatic massage)
- Transurethral resection
- Prostatic ischemia

Decreased in

- Castration
- Prostatectomy
- Radiation therapy
- Ejaculation withi 24 - 48 hours
- 5-alpha-reductase inhibitor reduces PSA by 50% after 6 months in men without cancer

Limitations

- PSA has been recommended by the American Cancer Society for use in conjunction with a DRE for early detection of prostate cancer starting at the age of 50 years for men with at least 10 year life expectancy
- PSA levels that are measured repeatedly over time may vary because of biologic variability where the true PSA level in a given man is different on different measurements.
- A change in PSA of >30% in man with a PSA initially below 2.0 ng/mL was likely to indicate a true change beyond normal random variation.

Dr. Purvish Darji
MD(Path)

Dr. Sanjeev Shah
MD Path

Dr.Yash Shah
MD Path

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Name : **Lyubochka Svetka**	**Lab Id : 02232160XXXX**	Client Name : Sterling Accuris Buddy
Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 12:06 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test	Result	Unit	Biological Ref. Interval
IgE *CLIA*	H **492.30**	**IU/mL**	**0 - 87**

IgE mediates allergic and hypersensitivity reactions. There is a significant overlap in total IgE between allergic and nonallergic individuals.

Interpretation:

- Increased In
  - Atopic diseases
    - Exogenous asthama in approximately 60% of patients
    - Hay fever in approximately 30% of patients and Atopic eczema
  - Influenced by type of allergen, duration of stimulation. Presence of symptoms, and hyposensitization treatment
  - Parasitic diseases (e.g. ascariasis, visceral larva migrans, hookworm disease, schistosomiasis, Echinococcus infestation)
  - Monoclonal IgE myeloma

- Decreased In

- Hereditary deficiencies
- Acquired immunodeficiency
- Ataxia-telangiectasis
- Non-IgE myeloma

Limitations:

- A normal level of IgE in serum does not eliminate the possibility of allergic disease.
- Serum total IgE levels for the majority of individuals with IgE-mediated disease can be expected to be elevated compared to the reference range for healthy adults. However, not all allergic patients exhibit elevated serum total IgE levels.
- Since not all atopic reactions are IgE-mediated, a total IgE result in the reference range should always be interpreted in light of other clinical observations.
- Heterophilic antibodies in human serum can react with the immunoglobulins included in the assay components causing interference with in vitro immunoassays.

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MD(Path)

Dr. Sanjeev Shah
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Patient Information	Sample Information	Client/Location Information
Name : **Lyubochka Svetka**	**Lab Id : 02232160XXXX**	Client Name : Sterling Accuris Buddy
Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 14:35 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Test	Result	Unit	Biological Ref. Interval
HIV I & II Ab/Ag with P24 Ag *Chemiluminescence*	0.070	S/Co	Non Reactive : <1.0 Reactive : >1.0
Interpretation	Non Reactive		
HBsAg *Chemiluminescence*	0.290	S/Co	Non Reactive : <1.0 Reactive: >1.0
Interpretation	Non Reactive		

Additional Information:

1. A NON REACTIVE result implies that no Anti HIV-1 or HIV -2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the "WINDOW PHASE" (before the development of detectable levels of antibodies).
2. A PROVISIONALITY REACTIVE / BORDERLINE REACTIVE result suggests possibility of HIV-1 or/and HIV-2 infection. However these results must be verified by confirmatory WESTERN BLOT / HIV PCR method before declaring the patient positive for HIV-1 or HIV-2 infection.
3. Very high levels of IgM Antibodies or Anti-HLA ABC and DR Antibodies can give false positive reaction.

**Pre & Post test counselling for HIV testing is responsibility of reffering Physician.

Dr. Siddharth Thummar
M.D. Pathology

Dr. Sanjeev Shah
MD Path

Dr.Yash Shah
MD Path

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Patient Information	Sample Information	Client/Location Information
Name : **Lyubochka Svetka**	**Lab Id : 02232160XXXX**	Client Name : Sterling Accuris Buddy
Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 14:16 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : EDTA Blood	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

HB Electrophoresis By HPLC

Instrument Name: BIORAD VARIANT - II Haemoglobin Testing System

Test	Result	Unit	Biological Ref. Interval
Hb A	L **84.4**	%	96.8 - 97.8
Hb A2	2.8	%	2.2 - 3.2
P2 Peak	5.5	%	
P3 Peak	5.2	%	
Foetal Hb	0.3	%	0.0 - 1.0
Interpretation	**Negative for typical beta thalassemia trait.**		

Interpretation:

- All results have to be correlated with age and history of blood transfusion if there is history of blood transfusion in last 3 months, repeat testing after 3 month from last date of transfusion is recommended.
- In case of haemoglobinopathy, parents or family studies and councelling is advised.
- This test detects beta thalassaemia and haemoglobinopathies, DNA analysis is recommended to rule out alpha thalassaemia and silent carriers.
- Linearity range of HbF is 1-40%, However, values in excess of the reportable range have been provided for ease of interpretation.
- Mild to moderate increase in fetal haemoglobiin can be seen in some acquired condition like pregnancy, megaloblastic anaemia, Throtoxicosis, Hypoxia, Chronic kidney disease, Recovering marrow, MDS, Aplastic anaemia, PNH, Medications (Hydrocyurea, Erythropoietin) ect.
- P3 window-Above 10% is often indicative of either denatured froms of hemoglobins or may suggest a possibility of abnormal haemoglobin variant. Hence, repeat analysis with fresh sample or DNA studies is advised.
- P2 Window-Above 10% is indicative of either glycated haemoglonin requring correlation with diabetic staus or may suggest a possibility of abnormal haemoglobin variant further DNA studies for confirmation.

Dr. Hardik Modi
Hematopathologist (G-18097)

Dr. Sanjeev Shah
MD Path

Dr.Yash Shah
MD Path

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Email: customer.care@sterlingaccuris.com | **Website:** www.sterlingaccuris.com

Patient Data

Sample ID: **022321600126**
Patient ID:
Name:
Physician:
Sex:
DOB:
Comments:

Analysis Data

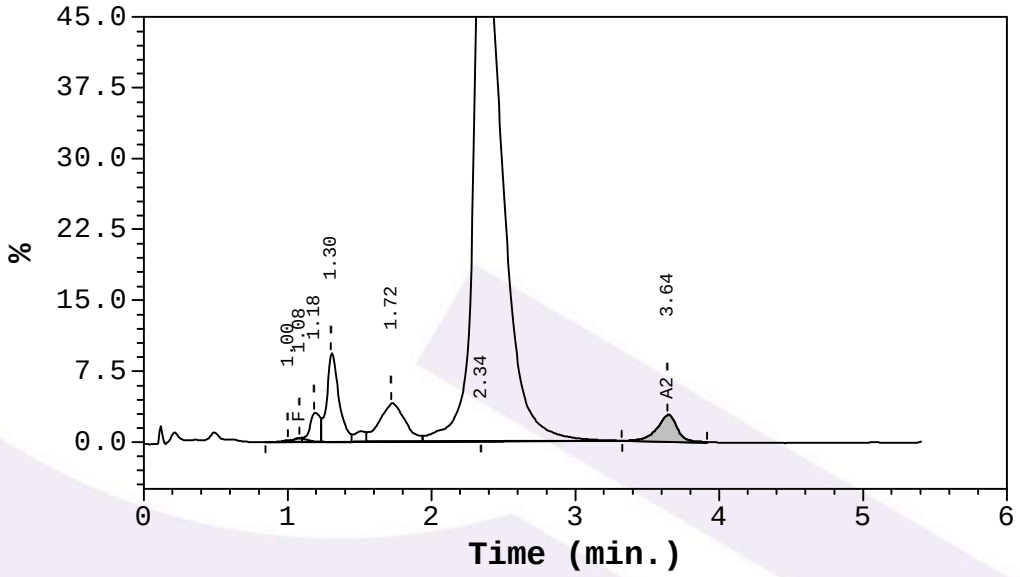
Analysis Performed: 02/20/2023 13:56:59
Injection Number: 2575
Run Number: 95
Rack ID: 0001
Tube Number: 2
Report Generated: 02/20/2023 14:04:40
Operator ID:

Peak Name	Calibrated Area %	Area %	Retention Time (min)	Peak Area
Unknown	---	0.1	1.00	2400
F	0.3	---	1.08	8321
Unknown	---	1.6	1.18	44109
P2	---	5.5	1.30	148807
P3	---	5.2	1.72	139183
Ao	---	84.4	2.34	2277592
A2	2.8	---	3.64	76999

Total Area: 2,697,411

F Concentration = 0.3 %
A2 Concentration = 2.8 %

Analysis comments:



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Sex/Age : **Male / 41 Y** 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:12 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Urine	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Test	Result	Unit	Biological Ref. Interval
Physical & Chemical (Dip strip) examination			
Colour	Pale Yellow		Pale Yellow
Clearity	Clear		Clear
pH *Double indicator*	6.0		4.6 - 8.0
Specific Gravity *Polyelectrolyte based reaction*	1.030		1.005 - 1.030
Urine Glucose *GOD-POD*	**Present (+)**		**Absent**
Urine Protein *Protein error of indicators*	Absent		Absent
Bilirubin *Diazo reaction*	Absent		Absent
Urobilinogen *Modified Ehrlich reaction*	Absent		Absent
Urine Ketone *Nitroprusside*	Absent		Absent
Nitrite *Nitrite reaction*	Absent		Absent
Microscopic Examination			
Pus Cells	1-2		Absent
Red Cells	Nil	/hpf	0 - 2
Epithelial Cells	1-2	/hpf	.
Casts	Absent	/hpf	Absent
Crystals	Absent	/hpf	Absent
Amorphous Material	Absent		

---------- End Of Report ----------

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M.D. Pathology

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